

Standard Operating Procedure

Code Blue / CPR Response

«Hospital Name» — Supports COP.5.a through COP.5.f

SOP No.	[Hospital to define]	Version	1.0
Effective Date	_____	Review Frequency	Annual, or on change
Prepared By	Quality Coordinator Name: _____ Signature: _____		
Approved By	Medical Superintendent Name: _____ Signature: _____		

1. Purpose

This SOP describes how «Hospital Name» responds immediately to a cardiac or respiratory arrest anywhere in the hospital, with a trained team, working equipment, and structured post-event review.

2. Scope

Applies to every area of the hospital — wards, OPD, diagnostic areas, and any location a patient, visitor, or staff member could require CPR.

3. Responsibility

First Responder (any staff member): starts BLS immediately on recognising the need — never waits for the code team.

Code Blue Team: responds to the call, leads advanced resuscitation, and records the event.

Post-Event Analysis Committee: reviews every event within the defined time frame.

4. Procedure

4.1 Immediate response

1. Start basic life support immediately on identifying a CPR-requiring condition — in every area, with no delay for any reason.
2. Call the Code Blue team per the hospital's activation process.

4.2 Team response

1. Respond per the assigned Code Blue role — team leader, airway, compressions, medications, documentation, as designated on the roster.
2. Use the defibrillator from the nearest accessible location without delay.
3. Confirm emergency medications and intubation equipment are available in the area — this is checked routinely, not only during an event.

4.3 Documentation

1. Record the event: personnel present, response timeliness, resource availability, and any barriers encountered.
2. For mock drills, record the same information as for actual events.

4.4 Post-event analysis and improvement

1. The multidisciplinary committee (physician/cardiologist, anaesthesiologist, code blue member, nurse) meets at least quarterly.
2. Analyse initiation time, team arrival time, resource availability, event sequence, coordination, and outcome, within the hospital's defined time frame after the event.
3. Complete CAPA within the defined time frame, and communicate findings back to the participants — closing the loop with the people who were actually there.

5. Records

Supported by all 9 records in the COP.5 Required Records pack.

6. Related Documents

- HCO_COP_5_v2_REWRITE_DRAFT — COP.5 policy

7. Review

Reviewed annually, and immediately after any CPR event with an unfavourable outcome linked to a process gap.

8. References

- American Heart Association. (2025). Guidelines for CPR and Emergency Cardiovascular Care. <https://cpr.heart.org/en/resuscitation-science/cpr-and-ecc-guidelines> — the immediate-response and team-role structure in this SOP is consistent with AHA resuscitation guidance.